BERNSTEIN
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U.S. Healthcare Services

US HC Svcs: The Policy Prescription - Healthcare Policy After the 2026 Midterms and the Road to 2028

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As we near July, the midterm elections and their impact on the market increases. In this note we outline our outlook for the midterms, healthcare policy direction after the midterms, and an early look at 2028 election cycle.

At this distance, based on the latest polls, it looks likely that Democrats will take control of the House, while the Senate remains a toss-up. What this means for policy - Democrats' main focus would likely be reversing declines in Medicaid and Marketplace enrollment. Policy outcomes should be broadly similar whether Democrats win just the House or both chambers of Congress, as both would likely require compromise to pass legislation and avoid vetoes. Some policies proposals will be political devises to contrast healthcare positioning in advance of the 2028 election.

Stock implications of the Midterms - A Democratic flip of the House would likely benefit Medicaid-focused insurers and hospitals by increasing the chances of restoring Medicaid funding and ACA enhanced subsidies. A potential method would involve delaying implementation of certain features of the elements OBBBA as part of a compromise during 27/28. This would reduce risk pool shifts which have worsened MLRs for Medicaid MCOs, stabilize Medicaid enrollment, and also mitigate some growth in bad debt at hospitals. We see actions against MA plans or PBMs as much less likely over this time period given the major policies enacted in these areas over the past few years.

2028 Politics and Policy, our early thoughts - While it is very early to identify front-runners, in the note we outline current likely candidates. Our main focus is on which policies are likely to be impacted by the 2028 election. We see policies as likely focusing on 1) coverage and reducing uninsured rates, 2) affordability, and 3) areas like vertical integration and AI. Overall, we see the policy environment as potential more positive for safety net MCOs and relatively neutral for the overall MCO, PBM complex. We see Medicare of All as unlikely to become policy, but expect that a Medicare For All candidate(s) will be featured within the Democratic primary and this may cause some volatility during 2027. We expect the most substantive changes might be in the areas of vertical integration restrictions.

We are hosting a teach-in next Thursday (6/18 at 11am ET) on Optum Health and value based care (Click here to register). This session will cover the Optum Health's business model and portfolio of businesses, clinical model and value proposition, an economic model for Optum Health looking at relevant subsegments, the outlook for margin recovery and growth, turnaround strategies and areas of policy focus.

Our recent Medicare Advantage teach-in addressed policy aspects and outlook for this line. Here is the recap note on the teach-in (**US Healthcare Services: A summary of our teach in on Medicare Advantage** *), along with the REPLAY and Slides.*

BERNSTEIN TICKER TABLE

			11 Jun 2026		TTM	Adjusted EPS				Adjusted P/E (x)		
			Closing	Price	Rel.							
Ticker	Rating	Cur	Price	Target	Perf.	Cur	2025A	2026E	2027E	2025A	2026E	2027E
AGL (agilon health)	M	USD	116.16	86.00	89.4%	USD	(24.46)	(4.81)	(4.52)	(4.7)	(24.2)	(25.7)
CNC (Centene)	O	USD	63.54	68.00	(8.0)%	USD	2.08	3.38	4.23	30.5	18.8	15.0
CI (Cigna)	O	USD	294.84	371.00	(29.3)%	USD	29.84	30.38	33.00	9.9	9.7	8.9
CVS (CVS)	O	USD	100.48	106.00	28.7%	USD	6.75	7.37	8.14	14.9	13.6	12.3
ELV (Elevance)	O	USD	399.18	424.00	(19.4)%	USD	30.30	26.93	28.88	13.2	14.8	13.8
HCA (HCA)	M	USD	378.51	413.00	(20.9)%	USD	28.21	30.44	33.14	13.4	12.4	11.4
HUM (Humana)	O	USD	368.69	425.00	34.1%	USD	16.60	9.12	15.41	22.2	40.4	23.9
MOH (Molina)	O	USD	193.20	208.00	(57.0)%	USD	11.03	5.35	9.65	17.5	36.1	20.0
UNH (UNH)	O	USD	405.55	492.00	4.5%	USD	16.35	18.86	21.99	24.8	21.5	18.4
SPX			7,394.30									

O - Outperform, M - Market-Perform, U - Underperform, NR - Not Rated, CS - Coverage Suspended

Source: Bloomberg, Bernstein estimates and analysis.

INVESTMENT IMPLICATIONS

We rate AGL Market-Perform with a target price of $86, CNC Outperform with a target price of $68, CI Outperform with a target price of $371, CVS Outperform with a target price of $106, ELV Outperform with a target price of $424, HCA Market-Perform with a target price of $413, HUM Outperform with a target price of $425, MOH Outperform with a target price of $208, and UNH Outperform with a target price of $492.

DETAILS

As we near July, the midterm elections and their impact on the market increases. In this note we outline our outlook for the midterms, healthcare policy direction after the midterms, and an early look at 2028 election cycle.

We believe the prospects of a potential Democratic victory in the House are modestly reflected in improved recent Medicaid stock performance (e.g. CNC and MOH), although this is not the sole catalyst. We note that hospitals would also be presumed beneficiaries of some of these potential policies and this does not seem to be providing much support to hospital stock prices yet.

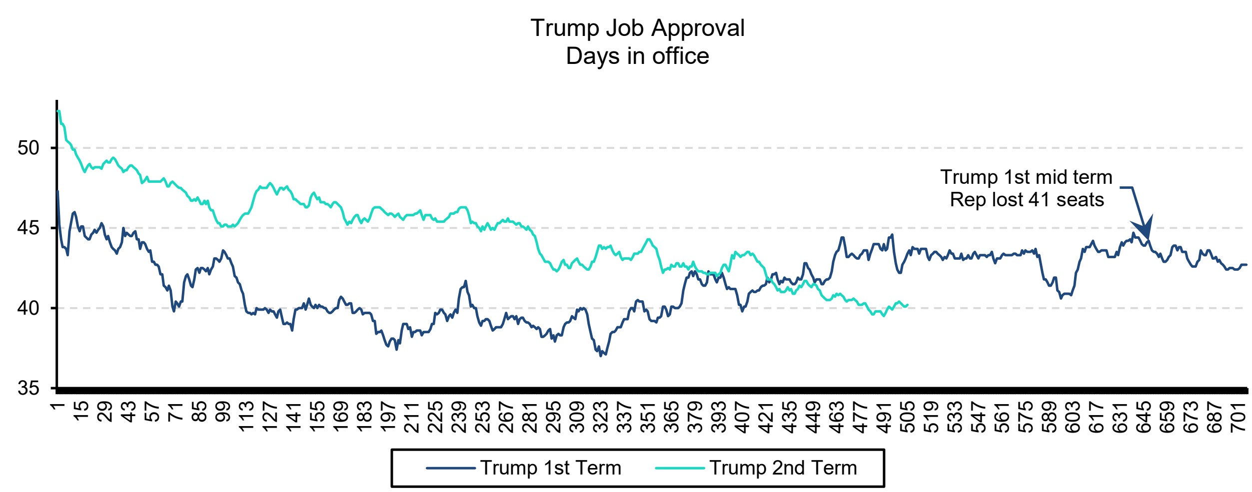
OUTLOOK FOR MIDTERMS

The political outlook, based on polls, suggests a Democratic flip of the House in the mid-terms, with around a 50/50 chance of flipping the Senate. This is a very volatile political environment with the current Iran conflict contributing to the volatility. Commentators expect a Democratic pickup of 15-20+ seats as the more likely range, although this is less than a traditional wave election. We are currently interpreting polarization of seats, coupled with redistricting as reducing the amount of turnover one might see in other wave type mid-terms. Likewise, the Senate is less certain to flip given that it will require two states which went 10+% for Trump to flip Democratic (OH, TX, AK etc.).

Trump's approval ratings

Trump approval ratings now stands at 40.2, a minor recovery from the lowest rating in his second term of 39.5 few days ago. The current rating has now slipped below the approval rating Trump had during his first term for the same days in the office. For historical context, Trump's approval rating is slightly below Obama at this point in his second term (43.1%) and above Bush (37.3%). Obama lost 13 seats, while Bush lost 32 seats during those 2nd mid-term.

EXHIBIT 1: **Trump's approval rating**

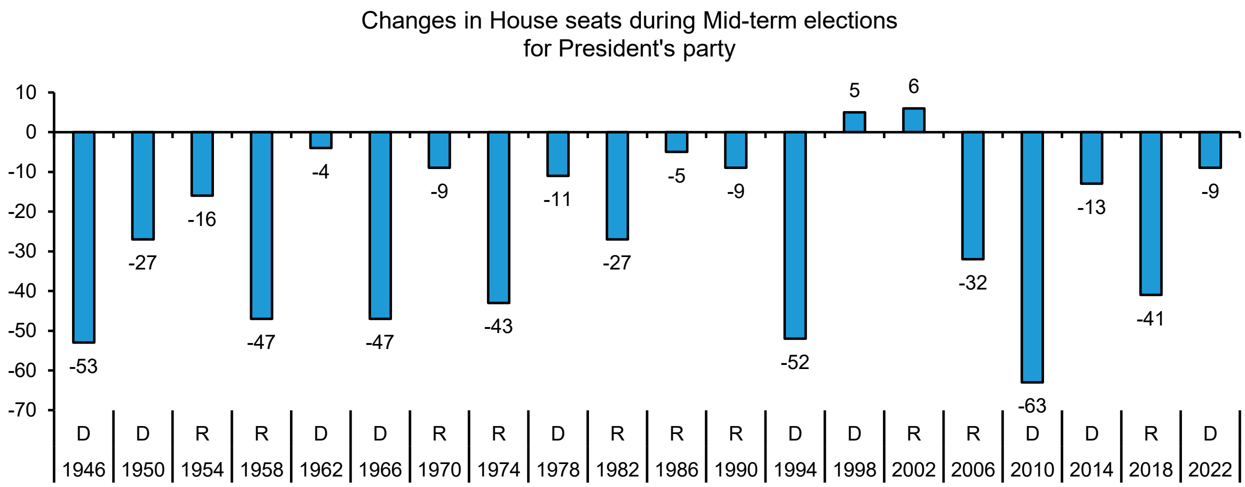


Source: RCP, Bloomberg, Bernstein analysis

2026 Midterm House elections

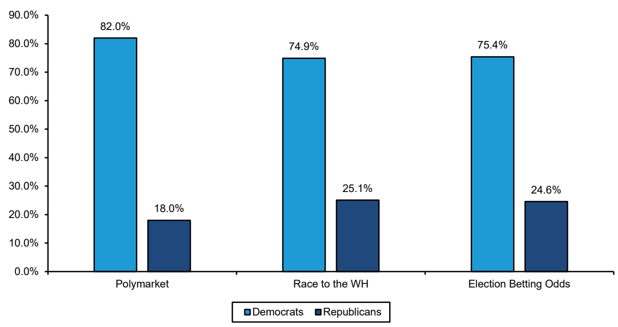
Republicans are expected to lose control of the House (Exhibit 5), as Republican currently have a small 5 seat edge (based on '24 vote) and the history of seat losses during midterms for the President's party. Redistricting complicates the equation, but most prediction markets expect Democrats to take control of the House. The margin of potential victory is less important in the House, but prediction markets expect around 15 seats. While this would seem a smaller pickup than other time periods given the President's low approval ratings, we see around 200 seats as roughly a floor for Republicans, based on polls, and therefore would not expect a much larger pickup by Democrats.

EXHIBIT 2: **Change in House seats in mid-term elections for President's party**



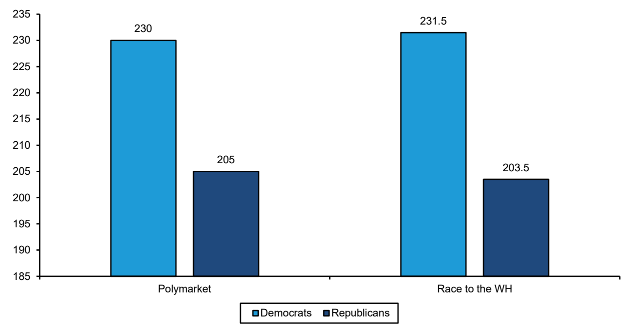
Source: Ballotpedia, US election results, Bernstein analysis

EXHIBIT 3: **Chance to Win House Majority**



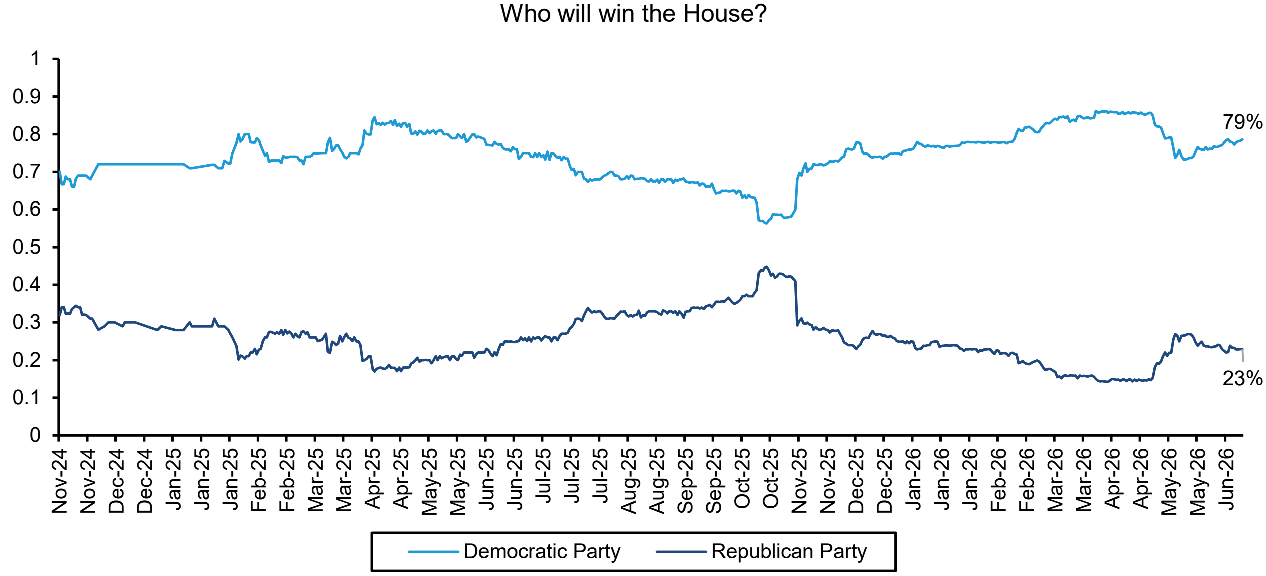
Source: Polymarket, Race to the WH, Election Betting Odds, and Bernstein Analysis

EXHIBIT 4: **Midterm House Seats Projection**



Source: Polymarket, Race to the WH, Election Betting Odds, and Bernstein Analysis

EXHIBIT 5: **Prediction market on who will will the US House?**



Source: Kalshi, Bernstein analysis

2026 Midterm Senate elections

35 Senate seats are up for election in 2026, including 33 regularly scheduled seats and 2 special elections. The two special elections are Marco Rubio (FL) and J.D. Vance (OH). Out of the 35, Democrats control 13 and Republicans have 22 seats currently. ME and NC are the states expected to most likely swing Dem, given ME voting for Harris in 24 and NC retirement of Tillis and a strong candidate in former Governor Cooper. The next two states are less likely, with OH and TX the more probable toss-ups. Movement of these states would signify a strong blue wave, which certainly is possible, but will be difficult to call all the way up to the election given that both states went for Trump by over 10 points. Currently, we see Republicans holding 50-51 seats, but 49 or lower is a possibility.

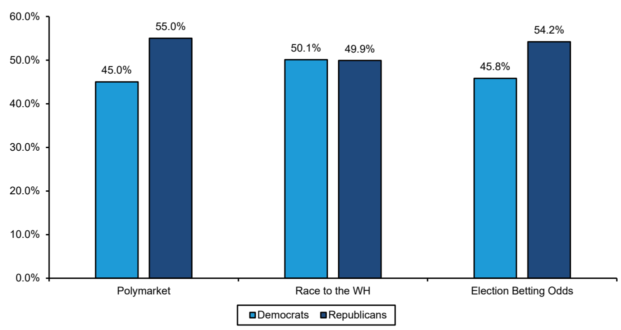
Prediction markets, such as Polymarket and Election Betting Odds, are predicting Republicans as slight favorites to retain their majority while Race to the WH has slight odds on Democrats gaining a majority. Kalshi has the most favorable odds for Republicans to retain the Senate, at 57%. Senate seat projections are closely contested as well, with most prediction markets expecting 51 Republican seats and 49 Democratic.

EXHIBIT 6: **Senate - Key races for 2026 midterms**

State	Party in Control of reelection seat	Incumbent running for Senate?	Margin of Victory for President			Margin for Senate Elections
Republican seats most at risk:			**2024**	**2020**	**2016**	**2020**
ME	Republican	Yes - Susan Collins	D +6.9%	D +9%	D +3%	R +8.6%
NC	Republican	No - Thom Tillis	R +3.2%	R +1.34%	R +3.7%	R +1.8%
OH	Republican	Yes - Jon Husted	R +11.2%	R +8.0%	R +8.5%	R +6.1%
IA	Republican	No - Joni Ernst	R +13.2%	R +8.2%	R +9.4%	R +6.5%
TX	Republican	Yes - John Cornyn	R +13.7%	R +5.6%	R +9.0%	R +9.6%
Democratic seats to watch:						
MI	Democrats	No - Gary Peters	R +1.4%	D +2.8%	R +0.2%	D +1.7%
GA	Democrats	Yes - Jon Ossoff	R +2.2%	D +0.2%	R +5.1%	D +1.2%
NH	Democrats	No - Jeanne Shaheen	D +2.8%	D +7.4%	D +0.4%	D +15.6%

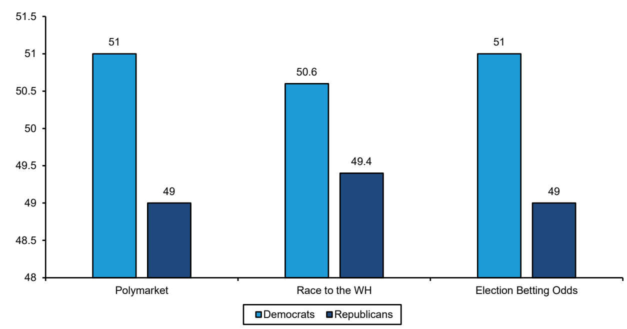
Source: Ballotpedia, Bernstein analysis

EXHIBIT 7: **Chance to Win Senate Majority**



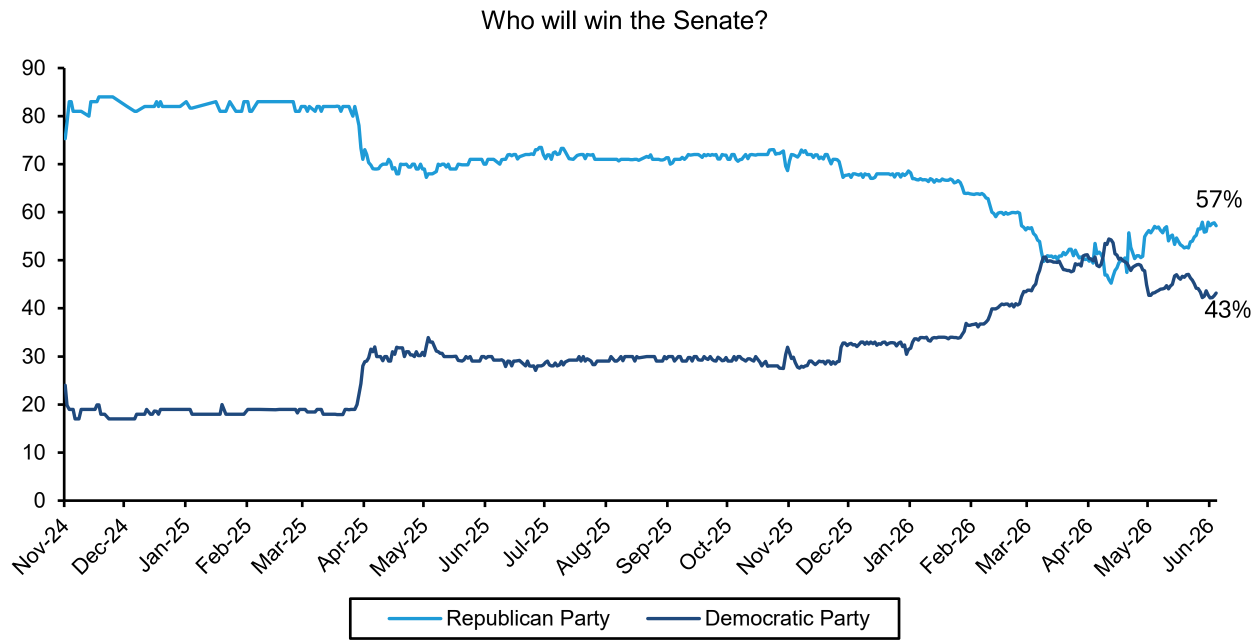
Source: Polymarket, Race to the WH, Election Betting Odds, and Bernstein Analysis

EXHIBIT 8: **Midterm Senate Seats Projection**



Source: Polymarket, Race to the WH, Election Betting Odds, and Bernstein Analysis

EXHIBIT 9: **Prediction market on who will win the US Senate**



Source: Kalshi, Bernstein analysis

POLICY IMPLICATIONS FROM MIDTERMS

The Democrats are making Medicaid and Marketplace cuts as a central theme in their campaigns, and we expect efforts to reverse these policies as a major focus for Democrats. The Senate Finance Committee led by Senator Wyden and several Senate Democrats released a letter in which they announced the healthcare reforms and policy changes they would go after.

Reversing Republican cost increases – Aimed at reversing the health care cuts enacted by Republicans which included refusal to extend premium tax credits. Democrats plan to restore the ACA safety net funding and develop policies to make enrollment easy and coverage affordable. They also mentioned intention to control annual spikes in deductibles and out-of-pocket expenses, introduce policies to enhance access to coverage for low-income population for non-expanded states via Medicare-type choices, eliminating junk insurance plans, and eliminating surprise tax bills.

Simpler access to insurance – Democrats will develop polices which will make getting insurance a one-stop shop. Their goal it to protect people from losing out on coverage due to costs or red tape. Policies will also be made for simplifying and standardizing plans for easy comparison for consumers.

Addressing corporate greed - They also mentioned holding “Big Insurance” accountable for practices which delay or deny access to care, making sure funds are used towards driving patient satisfaction and quality of care, eliminating the role of middlemen, and lowering the costs for consumers.

Our take: The first priority will be re-establishment of the enhanced Marketplace subsidies. These provide a clean policy and Republicans were in favor of a compromise during the recent showdown on this issue. We see funding for this type of improved subsidy which is tied to availability to HSA’s or expansion / increased flexibility on Bronze plans as a potential solution. This could still retain some of the reforms such as no zero dollar premium plans to combat fraud.

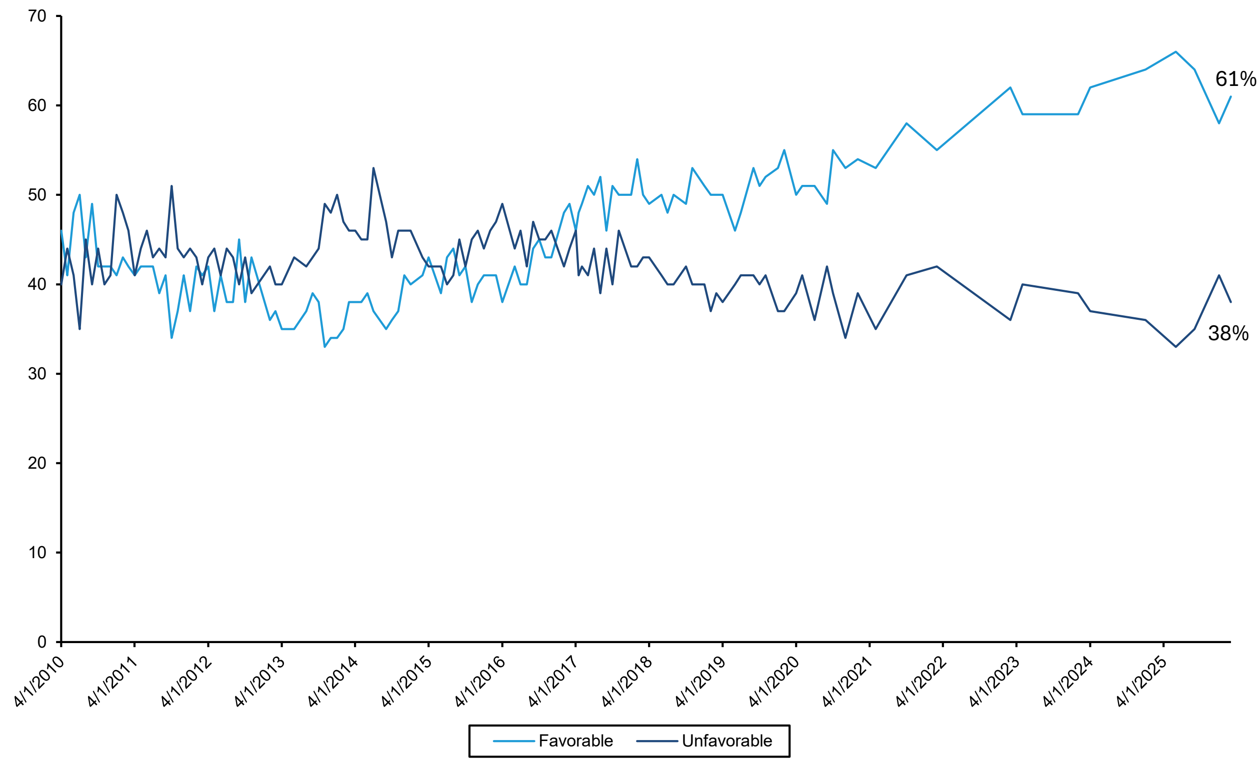
Medicaid moderation policies will be a focus but may be more difficult to find a compromise position. Republicans are likely to oppose provider tax / state directed payments policies which allow for states to provide less of the net costs of Medicaid. Work requirement refinements are a likely area of potential compromise, but full repeal of work requirements seems less likely. We see the most likely compromise position on Medicaid as being agreements to delay implementation of certain components. We see delays in implementation of provider tax safe harbor phase downs as a very likely target for compromise. This also is less effective of a pure political item, so it may be a more likely compromise item.

We expect the other major policy areas of focus to be more in the form of investigations and committee hearings, as opposed to likely compromise legislation. These would include hearing on drug pricing and vertical integration.

How can Democrats accomplish policy changes without a change in the Presidency? If Democrats have control of at least the House, they can leverage several critical must-pass bills to force legislation in case they win the House in the upcoming midterms. One example is the annual funding bill or series of bills, which could provide Democrats some leverage to extract a compromise. Another opportunity is the debt ceiling is another major must pass legislation event that might arise in mid-2027 with negotiations starting in early 2027. The OBBBA set the US debt ceiling in July 2025 at $41.1 trillion. Currently, the national debt is likely to be over $38 trillion by March 2026. The limit is likely to be reached in 2027 and without any "extraordinary measures" by the Treasury department, there can be a risk of default which puts pressure on the government to raise the ceiling. The Reconciliation bill could diminish some of this leverage, but we expect there will still be some must-pass bills which provide Democrats the opportunity to extract some compromises.

If Democrats have both the House and Senate, they are in a position to pass bills which President would need to veto or negotiate. This scenario is more likely to accomplish compromise policies, so prospects of a Senate victory by Democrats would likely be seen as more positive for Medicaid MCO and hospital stocks.

EXHIBIT 10: **ACA favorability as per KFF Health Tracking Polls**



Source: KFF Health Tracking Polls, Bernstein analysis

Stock Implications from Midterms

A Democratic flip of the House or House/Senate would have positive implications for Medicaid names (CNC, MOH) and hospitals (HCA). In our view, Democratic Representative would push to restore Medicaid funding and ACA Exchange enhanced subsidies to prior levels. A Democratic majority in the House would decrease the likelihood of further Medicaid funding pressure and increase likelihood of the restoration of Medicaid and Marketplace funding. Hospitals would benefit from a reduction in uninsured patients and the resulting decrease in uncompensated care and bad debt expense.

Historically, MA has been supported by Republican administrations and restricted by Democratic administrations, but we do not see MA being a policy point for Democrats during this cycle. We expect midterm results to be largely neutral to MA names (UNH, HUM).

In our view, the recent PBM reform (both PBM bill and FTC settlements) are clearing the policy hurdles for CI, CVS, and UNH. The PBMs have changed their business models and have largely removed the policy overhang moving forward. We do not view a Democratic flip of the House and/or Senate as a major threat to PBMs.

2028 ELECTION - WAY TOO EARLY THOUGHTS

Potential leading candidates

The 2028 election will be most impacted by the Presidential nominees and ultimate winner. We would expect that the most potential stock impact from a Medicare for All candidate on the Democratic side. We will discuss policy outlook below. We would expect the Democratic primary season to kick off in early 2027, similar to 2019, with Democrats eager to confront President Trump's policies and gain the mantle of leadership, particularly if Democrats take control of both the House and Senate. For Republicans, we would expect more of a stealth primary season during 2027, with actual campaigning kick off later in 2027.

Currently, prediction markets and polling suggests that the front-runners for the Democratic are Newsom, Harris, Buttigieg and Ocasio-Cortez. This list likely does not reflect a governor or outsider candidate or two that may gain traction during the pre-primary period. We would note that Ocasio-Cortez is the major Medicare For All candidate among the front-runners, while others leading candidates are more likely to focus on restoring and enhancing the ACA framework.

On the Republican side Vance and Rubio are the front-runners currently, but political commentators have cited DeSantis, possibly Trump Jr and another outsider (to the current Administration) candidate to be among the front-runners. These candidates are likely to range from populist to conservative Republican, and we would expect many of the Trump healthcare policy priorities to carry through to the next Republican nominee. The areas of potential change seem more likely on the populist side. We are watching for proposals on break-ups or restrictions on vertical integration as we see populists as possibly becoming the Trust Busters of the 21st century. We also could see expansion of safety net for the working class becoming a Republican proposal, especially from the JD Vance wing. We see employment uncertainty due to AI as being a potential issue for each party, and could see health insurance portability and/or expansion of Marketplace/ICHRA as being prominent themes.

EXHIBIT 11: **2028 Presidential nomination polls**

2028 Presidential nomination									
Democrats	**Harris**	**Newsom**	**Buttigieg**	**Ocasio-Cortez**	**Kelly**	**Shapiro**	**Beshear**	**Pritzker**	**Whitmer**
RCP Polling Average	27.1	17.9	13	11.9	6.5	6.3	4.2	3.8	2.3
Republicans	**Vance**	**Rubio**	**Trump Jr.**	**DeSantis**	**Kennedy Jr.**	**Haley**	**Carlson**	**Cruz**	**Ramaswamy**
RCP Polling Average	38.3	24.1	9.3	8.8	4	4	3.3	2.8	2.5

Source: RCP, Bernstein analysis

Policy focus for 2028 election (early thoughts)

We see health insurance coverage access and funding as an important element of the 2028 election policies. We expect recent Trump policies will have increased uninsured rates toward 9+% by 2028, with elimination of enhanced Marketplace subsidies and Medicaid work requirements visible examples.

We expect Democratic candidates will focus on three versions of coverage expansion - 1) restoring peak ACA coverage, 2) expanding off existing programs to achieve universal coverage, 3) Medicare for All.

The Restoration of Peak ACA Coverage would likely be achieved through the return of enhanced subsidies in Marketplace, coupled with elimination of work requirements for Medicaid and reduced verification policies. The full proposal could attempt to increase Marketplace coverage back to 24m people while COVID era Medicaid policies could drive Medicaid enrollment back toward 92m, a net increase of 25 million between the two programs. This would likely require mainly federal funding, so a repeal of provider tax restrictions could be apt of this, or a restructuring of the program to allow for consistent but more generous federal funding across all programs. One area to watch of rin this category would be Democrats seeking to restrain spending by differentiating between for profit and not for profit hospitals and providers in state directed payments, perhaps with a requirement that hospitals participate in Medicaid.

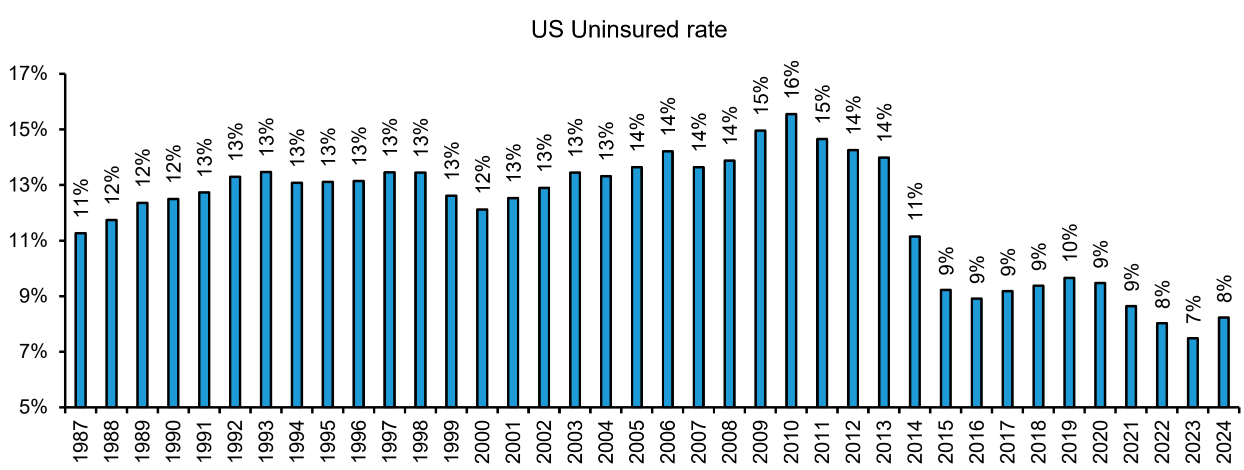
Universal coverage is a theoretical policy position which might build off the Peak ACA proposal above. Our expectation is that this type of proposal would broaden and expand access to the Marketplace by allowing small employers to buy into Marketplace, allow employees and dependents to have portability of employer health insurance which could allow for Marketplace coverage, and tie Marketplace reimbursement to government reimbursement rates (e.g. 120% of Medicare reimbursement.) This would be similar to the public option approach in Colorado, which created a “public option” which is provided by MCOs and achieves these aims above.

This universal coverage objective might also be a Republican policy focus in 2028 as Republicans seek to solidify their shift to a workers party. We think employer health insurance portability and easier access to Marketplace are policies they may target, with ICHRA being an important vehicle or early version of a new model. We see employment uncertainty in the era of AI job losses (or concerns about potential job losses), as a theme which both parties will seek to address.

We see Medicare for All as less likely than in prior cycles. Medicare Advantage is more firmly entrenched, now representing ~55% of Medicare enrollment vs. ~1/3 during earlier reform debates. Additionally, public sentiment has shifted more favorably towards ACA, which we believe makes the elimination of the ACA framework, along with employer health insurance, a less likely attractive policy aim. We expect a Democratic push to restore the uninsured rate to around the 5% level, from what we see could drift towards a potential 9% by 2028.

The Democratic shift in priorities from Medicare for All to expansion and accessibility of coverage through ACA can be observed in the shifting platforms of Democratic candidates. For example, Sherrod Brown (D-OH) served as a US representative from 1993-2007 and a US Senator from 2007-2025. He is currently challenging incumbent Jon Husted (R-OH) for the US Senate position in the Senate special election in Ohio. Rep. Brown was a proponent of Medicare for All early in his career, co-sponsoring the Expanded Medicare for All Act in 2006. His position moderated in subsequent years. In 2019, he pushed back against Medicare for All and co-introduced the Medicare at 50 Act which keeps ACA and private insurance intact but allows buy-in to Medicare for ages 50-64. He also co-introduced the CHOICE Act, an initiative which would add a publicly operated health insurance option to the Affordable Care Act’s individual marketplaces. It would create a public health insurance option subject to the same requirements that apply to other plans offered on Affordable Care Act exchanges, and it would offer the same tax credits available to individual marketplace consumers.

EXHIBIT 12: **Uninsured rate in the US**



Source: NHE, FRED, Bernstein analysis

The other major areas of policy focus are likely to be consumer affordability, the deficit and the costs of entitlement programs, and vertical integration.

In terms of consumer affordability we see drug pricing and hospital prices as the major areas of focus. MA overcoming has been largely addressed by v28 and recent other rate adjustments, and we expect this to be less of an issue by 2028. Likewise, we see PBM reform as having effectively addressed most PBM issues, and thus the focus will shift to drug pricing perhaps contrasting those costs with non-US prices. Solutions may attempt to address the core cost problems and focus on solutions such as value-based care and AI to increase supply of providers and reduce unit costs. More likely, policy proposals will focus on reducing out of pocket expenses for consumers (which will increase the need for MCO and VBC methods to address cost trends). Out of pocket limits could be for specific categories like drug spending / MFN and even specific products like GLP1s. Also, we could see interest in expanding out of pocket limits in employer health insurance to address the growth of high deductible plans. This could fuel the shift to defined contribution type health coverage which leverages Marketplace.

Entitlement program cost is an issue, but it is less likely to be a major poetical topic for 2028. The most powerful approaches which could be introduced could include mandatory enrollment in MA for those turning 65, but at a reduced payment level, and block grants to states for Medicaid. Medicare for All will also be proposed for this topic, with two major issues - converting all reimbursement to Medicare levels would destabilize the entire hospital / provider sector and the hidden tax of high reimbursement by employers would become an explicit tax which taxpayers would see. We see the more likely solutions being the reform of Medicare to be a MA only program going forward with existing members grandfathered, and this taking place as part of Medicare reform when the trust fund runs out. Likewise, further reform of Medicaid federal vs state funding is likely to take place during a broader reform of coverage and to be included, not to be the driver of that reform.

We see vertical integration as the sneaky issue which might gain traction on both sides of the aisle in 2028. We see the most natural issue as being dividing insurer vs provider - meaning PBMs and dispensing pharmacy being separated and then managed care companies and care providers including value-based care companies, as being separated. The PBM / specialty pharmacy split is being tested at a state level and could be more readily implemented at a federal level, likely through a series of spin-offs of the major specialty pharmacies. The split of MCOs and providers is more difficult, in particular due to the fully integrated models like Kaiser Permanente. We see the operational splits of UNH/Optum Health, CVS/Oak Street and Minute Clinic, and Humana / Centerwell Provider as being achievable as those providers are topically servicing multiple MCOs.

We see the more likely way this would be achieved is through new regulation to “stack MLRs” or treat margins achieved in subsidiaries as counting toward minimum MLRs. This policy is illogical, in that it penalizes MCOs while separate companies would still be able to earn margins at each level of the business model. Nonetheless, this approach would cause MCOs to divest or spin out subsidiary businesses. We see the three main categories of businesses as being MCO, cost containment businesses including PBM, and providers such as value-based care, physician practices and hospitals. We see cost containment businesses as naturally being coupled with MCOs, but this type of policy could cause the separation of these functions.

We see UNH and CI as being well situated for this policy direction because the units to be separated are large and leading players in their sectors. We see emerging businesses such as Oak street, Centerweel provider, ELV PBM (CarelonRx) as beng negatively impacted as they would become standalone competitors without the benefit of the larger MCO.

2028 Senate Election

We would note that the 2028 Senate map is better positioned for Republicans - both parties have 4-5 seats which are in competitive or somewhat competitive markets. Democrats have 4 seats which they will defend which went for President Trump in the 2024 election, while Republicans do not have seat among these most competitive states which had gone for Harris in 2024. That said, it would seem that WI and NC would be the Republican seats most at risk, while GA, PA, AZ and NV would be the Democratic seats most at risk.

The outcome of the Senate in 2028 is important in that the biggest policy changes would occur with a sweep of House, Senate and Presidency. It would seem policy would be more likely to occur after 2028, the filibuster is more likely to be ended or changed in 2028 given stated Democratic desires to end the filibuster and the likelihood that both Republican and Democratic centrists are the most at risk candidates in 2026 and 2028.

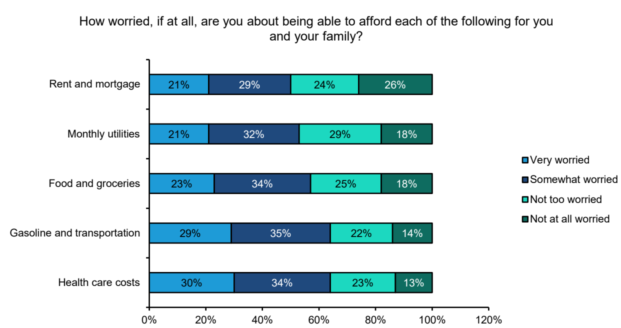
EXHIBIT 13: **2028 Key Senate elections**

State	Senate 2028 Pre-election incumbent	Party	Margin of victory Senate 2022, %	Margin of victory Presidential election 2024, %	State Presidential voting				
					2008	2012	2016	2020	2024
Nevada	Catherine Cortez Masto	D	0.8%	R +3.10%	D	D	D	D	R
Wisconsin	Ron Johnson	R	1.0%	R +0.86%	D	D	R	D	R
Georgia	Raphael Warnock	D	2.8%	R +2.20%	R	R	R	D	R
North Carolina	Ted Budd	R	3.2%	R +3.21%	D	R	R	R	R
Arizona	Mark Kelly	D	4.9%	R +5.5%	R	R	R	D	R
Pennsylvania	John Fetterman	D	4.9%	R +1.71%	D	D	R	D	R
Ohio	J.D. Vance	R	6.1%	R +11.2%	D	D	R	R	R
Alaska	Lisa Murkowski	R	6.3%	R +13.1%	R	R	R	R	R
New Hampshire	Maggie Hassan	D	9.1%	D +2.78%	D	D	D	D	D
Utah	Mike Lee	R	10.4%	R +21.6%	R	R	R	R	R

Source: Ballotpedia, Bernstein analysis

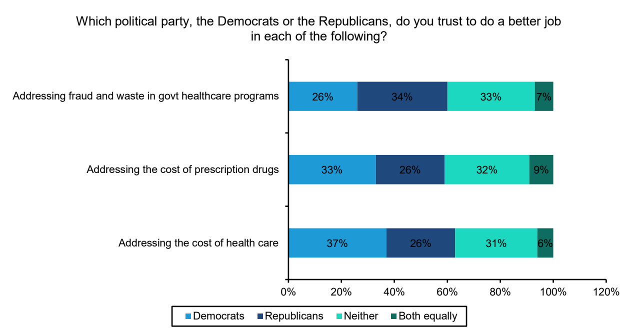
Healthcare cost is a significant concern for voters. A May 2026 study by KFF found that voters are equally concerned about healthcare costs and transportation costs, which was higher than concerns around groceries, utilities, and rent. Voters trust Democrats to address the costs of prescription drugs and healthcare over Republicans (33% to 26% and 37% to 26%, respectively). Healthcare reform, economy, and inflation continue to be major factors to voters. In a December 2025 survey, immigration was the highest agenda item followed by the economy, health care reform, cost of living, and inflation.

EXHIBIT 14: **Cost concerns**



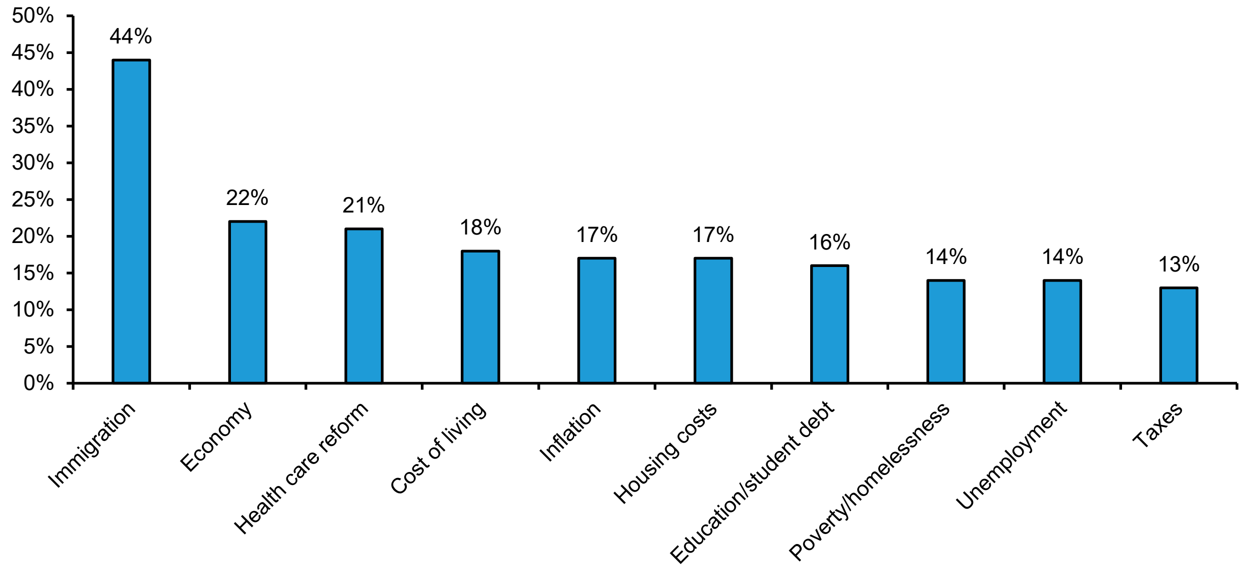
Source: KFF 2026 study, Bernstein analysis

EXHIBIT 15: **Healthcare cost concerns**



Source: KFF 2026 study, Bernstein analysis

EXHIBIT 16: **Dec 2025 survey of top 10 public concerns -% of respondents mentioning each issue**



Source: AP - NORC Center for Public Affairs Research at Univ of Chicago, Bernstein analysis

Stock implications - We expect MCOs will see volatility as Medicare For All is raised as a potential policy, but any progress by non-Medicare For All candidates and/or moderation in the application would be positives for MCOs. The Sherrod Brown example above is an interesting case study of how candidates may evolve in the 2028 era when ACA coverage has been rolled back and thus progress to reattain past coverage levels may be more politically acceptable. We would see Medicaid/ Marketplace MCOs and to a lesser extent hospitals as well positioned for these expansions of coverage policies.

Vertical integration and "Too big" policy focus - we believe that UNH would actually perform better in a policy environment which required the separation of provider and insurance businesses, and thus see it as a long term beneficiary, although it would see volatility initially. We would see VBC and AI as major opportunities for a standalone Optum. We also see CI as performing well in that environment, with the specialty pharmacy being separable and value enhancing without major earnings power challenges. Conversely, we see ELV as being negatively impacted if the potential to a value-based care business is no longer a cross-sale or synergistic opportunity. Likewise, CVS would see a high degree of disentanglement in this scenario and would be more pressured.

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